

ICD-0-3

path: carcinoma papillary/follicular + columnar 8344/3
(code to highest)

CACF: carcinoma, papillary, thyroid 8240/3
site: thyroid, NDS C73.9 4/26/11

Surgical Pathology Consultation Report

Patient
Name:

MRN:

DOB:

Gender: M

HCN:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #:

Collected:

Received:

Reported:

Specimen(s) Received

1. Parathyroid: right lower QS
2. Thyroid :Total Thyroidectomy left paratracheal node stitch marks left upper pole.

Diagnosis

1. No pathological diagnosis: Parathyroid (right lower) biopsy
2. Multifocal papillary thyroid carcinoma, dominant widely invasive mixed follicular and columnar cell variant with focal tall cell change and Warthin-like component, 1.7 cm, right; severe chronic lymphocytic thyroiditis and branchial cleft-like cyst, 0.2 cm: Thyroid
No pathological diagnosis: Parathyroid, right perithyroidal
No pathological diagnosis: Lymph nodes, 2, isthmus
No pathological diagnosis: Lymph node, left (paratracheal)
- Total thyroidectomy with left paratracheal dissection specimen

Synoptic Data

Procedure:

Received:

Specimen Integrity:

Specimen Size:

Other:Total thyroidectomy with left paratracheal neck dissection

Fresh

Intact

Right lobe:

5.0 cm

2.7 cm

1.6 cm

Left lobe:

5.9 cm

2.5 cm

1.3 cm

Isthmus +/- pyramidal lobe:

2.8 cm

1.2 cm

0.9 cm

Additional specimen: left paratracheal neck dissection

1.7 cm

1.2 cm

0.5 cm

Specimen Weight:

Tumor Focality:

25.5 g

Multifocal, bilateral

Multifocal, midline (isthmus)

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	KML 4/25/11	
Date Reviewed:	4/26/11	

UUID:013B2003-A884-40A0-8D85-8EE9D4DB960C
TCGA-EM-A22M-01A-PR

Redacted



----- DOMINANT TUMOR -----

Tumor Laterality: Right lobe
Tumor Size: Greatest dimension: 1.7cm
Additional dimension: 1.3 cm
Histologic Type: Papillary carcinoma, columnar cell variant
Papillary carcinoma, follicular variant
Papillary carcinoma, other variant: focal tall cell change and Warthin-like component.
Follicular architecture
Classical cytomorphology
Oncocytic or oxyphilic cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Distance of invasive carcinoma to closest margin: 0.1mm
Tumor Capsule: None
Tumor Capsular Invasion: Present, extent widely invasive
Lymph-Vascular Invasion: Not identified
Perineural Invasion: Not identified
Extrathyroidal Extension: Not identified

----- SECOND TUMOR -----

Tumor Laterality: Left lobe
Tumor Size: Greatest dimension: 1.0cm
Histologic Type: Papillary carcinoma, classical (usual)
Papillary carcinoma, follicular variant
Classical (papillary) architecture
Follicular architecture
Classical cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Tumor Capsule: Partially encapsulated
Tumor Capsular Invasion: Present, extent minimal
Lymph-Vascular Invasion: Not identified
Perineural Invasion: Not identified
Extrathyroidal Extension: m (multiple primary tumors)
TNM Descriptors: pT1b: Tumor more than 1 cm but not more than 2 cm in greatest dimension, limited to the thyroid
Primary Tumor (pT): pN0: No regional lymph node metastasis
Regional Lymph Nodes (pN): Number of regional lymph nodes examined: 3
Number of regional lymph nodes involved: 0
Distant Metastasis (pM): Not applicable
Additional Pathologic Findings: Thyroiditis, advanced
Parathyroid gland(s), within normal limits
Other: additional multiple papillary microcarcinomas, dominant 0.7 cm; and branchial cleft-like cyst in the right lobe
Ancillary Studies: Type: IHC
Results: The dominant tumor is positive for HBME-1 (membranous and apical), cytokeratin 19 (membranous) and galectin-3 (cytoplasmic and focal nuclear).

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Comment

The dominant tumor is a widely invasive mixed follicular and columnar cell variant with focal tall cell change and Warthin-like component. The focal tall cell change and Warthin-like component represent less than 10% of the tumor area. The dominant tumor is strongly positive for HBME-1 (membranous and apical), cytokeratin 19 (membranous) and galectin-3 (cytoplasmic and focal nuclear).

The dominant tumor also exhibits a very prominent lymphoplasmacytic background and the infiltrate shows a reactive staining pattern for kappa and lambda, and is composed of a mixed population of predominantly IgG and IgA cells. These cells are present at the interface with tumor and background thyroid but do not form definitive lymphoepithelial lesions with obliteration of epithelial islands. In summary, the features are of a reactive immune infiltrate.

by:

Electronically verified

Clinical History

Papillary thyroid ca

Gross Description

1. The specimen labeled with the patient's name and as "Parathyroid right lower QS" consists of a piece of soft tissue that is not weighed and measures 0.5 x 0.2 x 0.2 cm.

1A frozen tissue resubmitted

2. The specimen labeled with the patient's name and as "Total thyroidectomy left paratracheal node stitch marks left upper pole" consists of a thyroid gland that is received oriented with a suture and weighs 25.5 g. The right lobe measures 5.0 cm SI x 2.7 cm ML x 1.6 cm AP, the left lobe measures 5.9 cm SI x 2.5 cm ML x 1.3 cm AP and the isthmus measures 2.8 cm SI x 1.2 cm ML x 0.9 cm AP. There is a piece of adipose tissue at the inferior aspect of the left lobe that measures 1.2 x 1.7 x 0.5 cm. The external surfaces have fibrous adhesions and are painted with silver nitrate. No parathyroid glands are identified grossly. Two query lymph nodes measuring 0.7 cm and 1.0 cm are identified grossly in the adipose tissue at the inferior aspect of the left lobe. Both lobes and the isthmus contain multiple poorly delineated nodules that measure from 0.7 to 1.7 cm. The largest nodule in the isthmus contains a focal area of calcification. The remainder of the thyroid tissue is pale pink-tan. Sections of a nodule from each lobe and normal tissue are stored frozen. Representative sections of the remainder of the specimen are submitted as follows:

2A-F right lobe, superior to inferior

2G-I isthmus, in toto, except a focal area of calcification

2J-N left lobe, superior to inferior

2O 2 lymph nodes

Quick Section Diagnosis

1. Parathyroid, right lower: parathyroid tissue present.

Slide received ; reported

Microscopic Description

The dominant tumor is positive for HBME-1 (membranous and apical), cytokeratin 19 (membranous) and galectin-3 (cytoplasmic and focal nuclear). The prominent background plasma cell infiltrate shows a reactive staining pattern for kappa and lambda, and is composed of a mixed population of predominantly IgG and IgA cells.
